

PROJECT REPORT

Cancer Classification and Prediction Using Neural Networks

Deep learning for breast-tumour classification on the Breast Cancer Wisconsin (Diagnostic) dataset

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EXECUTIVE SUMMARY

A neural network that classifies tumours with 96.49% test accuracy

A Sequential neural network (30 → 16 → 8 → output) was trained on quantitative cell-nucleus measurements from the Breast Cancer Wisconsin (Diagnostic) dataset to classify tumours as benign or malignant. Trained for 10 epochs with the Adam optimiser, the model comfortably exceeded the 90% accuracy target set for the project and generalised well from training to unseen data.

The system was further validated by predicting on new, unseen patient measurements — correctly identifying tumour status with high confidence — confirming its viability as a decision-support proof of concept.

96.49%

Test-set accuracy

98.29%

Training accuracy

569

Patient records

30

Input features

Why automate tumour classification?

Conventional diagnosis depends on a pathologist examining tissue samples — a process that is slow, demands scarce specialist expertise, and carries inherent inter-observer variability. As case volumes rise, these constraints become a bottleneck in the diagnostic pathway.

1

Accuracy above 90%

Distinguish benign from malignant tumours reliably enough to support clinical decisions.

2

Seconds, not hours

Return a result far faster than manual slide review.

3

Consistent criteria

Apply identical logic to every case, removing observer-to-observer variability.

4

Minimal false negatives

Keep missed malignancies — the costliest error — to a minimum.

Where this project sits in prior work

Machine learning in healthcare

A decade of rapid progress in deep learning has produced diagnostic systems rivalling specialist clinicians on narrow tasks such as skin-lesion classification and diabetic-retinopathy screening. Performance tracks data quality and quantity more than model complexity.

Neural networks & backpropagation

Layers of interconnected units compute weighted sums through non-linear activations; backpropagation computes gradients and an optimiser updates weights. Adam — adaptive per-parameter learning rates — is now a standard, low-tuning choice.

Prior work on this dataset

Used as a benchmark since the early 1990s, when introduced alongside a linear-programming diagnosis approach. SVMs, logistic regression, random forests, and neural networks have all reported 94–98% accuracy — suggesting the classes are well separated and preprocessing matters more than classifier choice.

The role of feature scaling

Features span very different ranges — mean area in the thousands, mean smoothness under 0.2. Without standardisation, gradient descent is dominated by large-magnitude features. Rescaling to zero mean and unit variance is a requirement, not a refinement.

What the project set out to do

OBJECTIVES

- Classify breast tumours as benign or malignant
- Achieve test-set accuracy above 90%
- Implement a complete preprocessing pipeline, incl. standardisation
- Evaluate using accuracy, loss curves, and error analysis
- Build a working prediction system for new patient data
- Demonstrate that the model generalises rather than overfits

SCOPE

Covers the full ML workflow — loading data, exploratory analysis, quality checks, train/test split, standardisation, network design and training, evaluation, and prediction on new input. Deliberately limited to binary classification on the Wisconsin dataset; multiclass subtyping, imaging integration, and clinical deployment are future work.

DELIVERABLES

- **Source code:** Jupyter Notebook (.ipynb) and Python files
- **Documentation:** This project report
- **Evidence:** Screenshots of execution, training output, results
- **Model:** Trained Keras Sequential network

DATASET DESCRIPTION

Breast Cancer Wisconsin (Diagnostic) dataset

569 patient records, 30 numeric features derived from ten cell-nucleus measurements (radius, texture, perimeter, area, smoothness, compactness, concavity, concave points, symmetry, fractal dimension) — each recorded as mean, standard error, and worst value. An explicit null check returned zero missing values across every column, so the full sample was available for training and testing.

569

Total samples

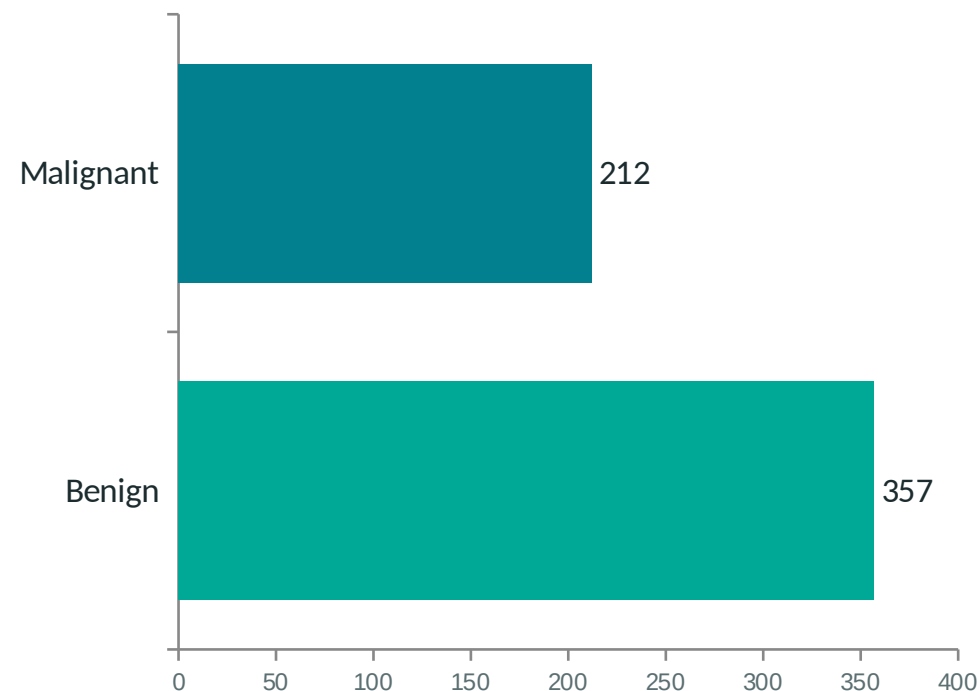
30

Numeric features

0

Missing values

CLASS DISTRIBUTION (569 SAMPLES)

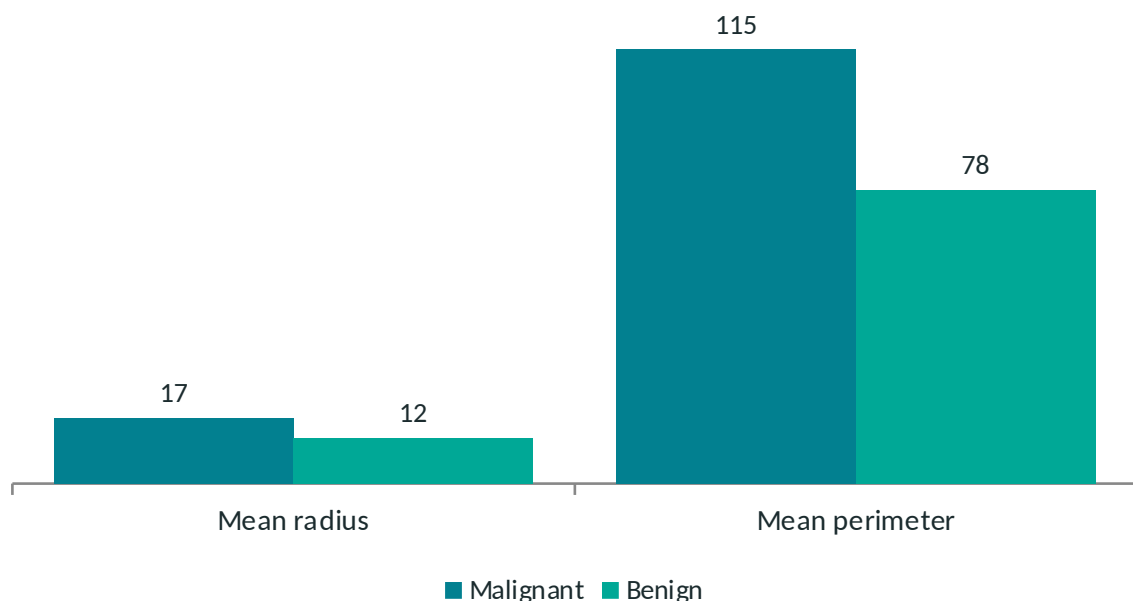


Benign: 357 (63%) • Malignant: 212 (37%) — a mild imbalance that trains well without resampling.

Classes are clearly separable in feature space

Grouping the data by label and computing feature means reveals a wide gap between the two classes. Malignant tumours show substantially higher mean radius, perimeter, and area — this separation is what makes the classification problem tractable for a neural network.

MEAN RADIUS & PERIMETER, BY CLASS



MEAN AREA, BY CLASS

Malignant

978.38

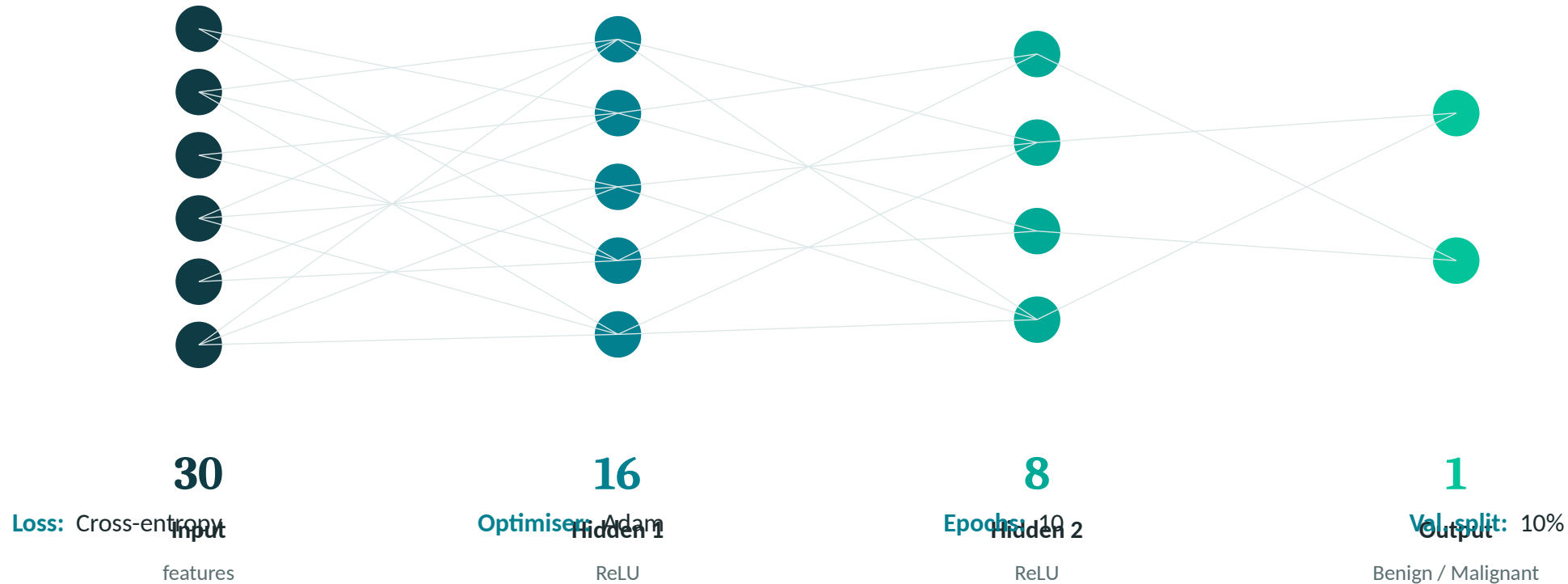
Benign

462.79

Malignant tumours average roughly 2.1× the mean area of benign tumours — a difference too large for the shared axis above.

A funnel-shaped feedforward network

Layer sizes narrow progressively from 30 → 16 → 8, compressing the input into an increasingly compact representation that retains diagnostic signal while discarding noise.



From raw data to a trained model

Python 3.12 • TensorFlow / Keras • NumPy & Pandas • scikit-learn • Matplotlib • Google Colab

1

Load & inspect

Load via scikit-learn into a Pandas dataframe; confirm 569 rows $\times$ 31 columns, no nulls.

2

Train / test split

80/20 split with a fixed random state $\rightarrow$ 455 training, 114 test samples.

3

Standardise

Fit StandardScaler on training data only, then transform both sets — avoids leakage.

4

Train the network

Fit for 10 epochs, Adam optimiser, 10% validation split, monitoring accuracy.

5

Evaluate

Score on the held-out test set; plot accuracy and loss curves.

6

Predict

Feed new patient measurements through the same fitted scaler and model.

Data preparation, straight from the notebook

Screenshots from the executed Google Colab notebook, confirming the dataset shape, the missing-value check, and the train/test split described in the methodology.

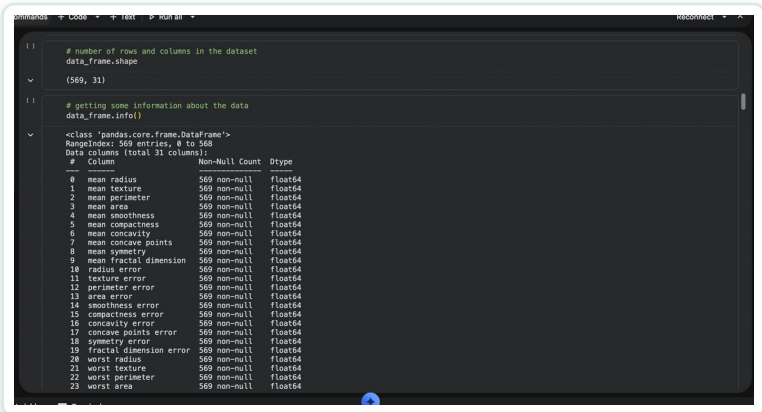


Fig. — dataset shape & dtypes (569 × 31)

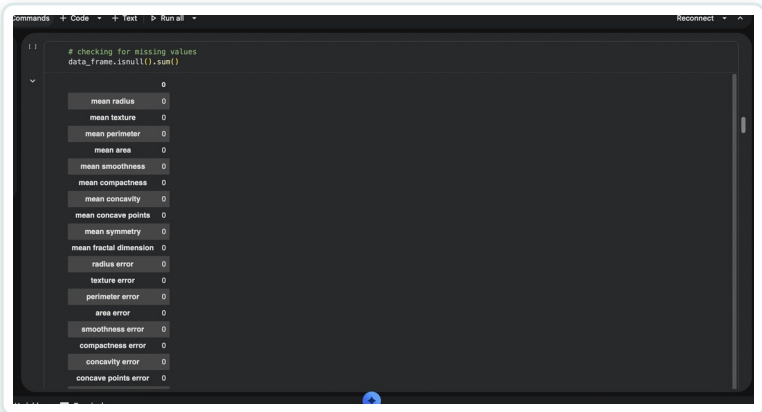


Fig. — null check, zero missing values

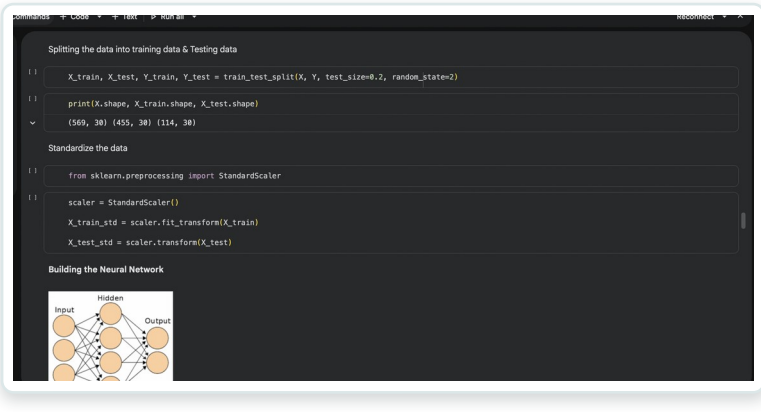
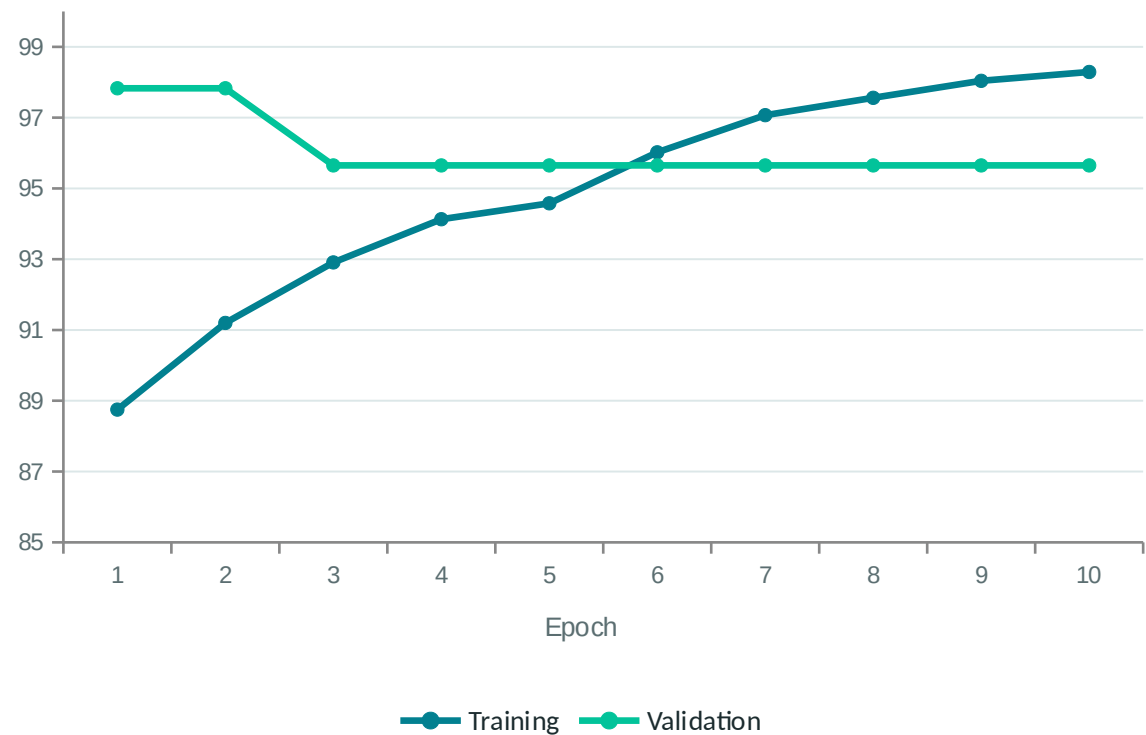


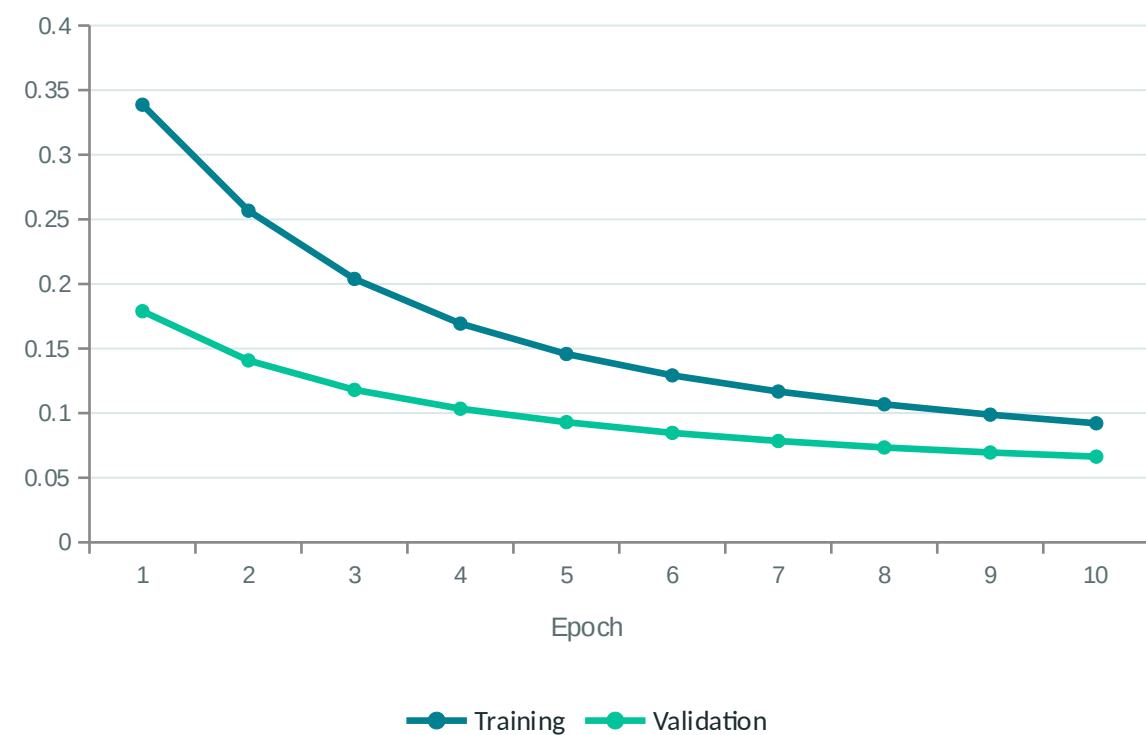
Fig. — 80/20 train/test split & standardisation

Accuracy climbs, loss falls, over 10 epochs

ACCURACY (%)



LOSS



Validation loss declines steadily throughout — no upward turn — indicating the model is not overfitting within these 10 epochs.

Training run, straight from the notebook

The raw epoch-by-epoch Keras output and the matplotlib accuracy/loss plots generated directly from the training history object.

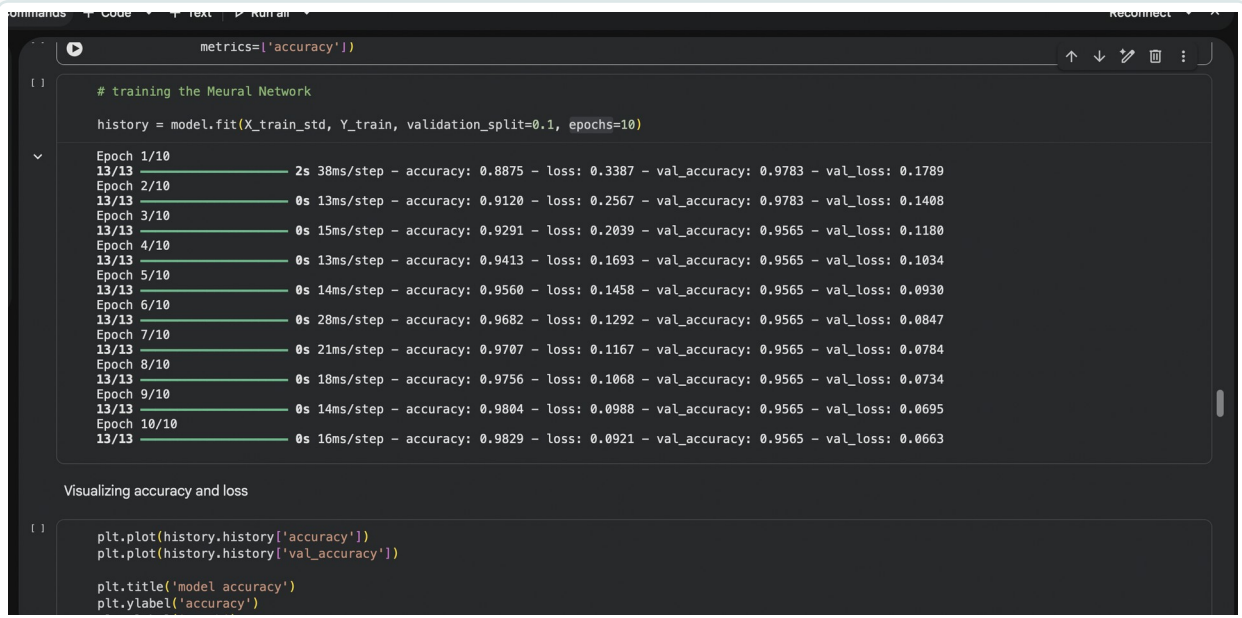


Fig. — epoch-by-epoch training log (10 epochs)



Fig. — training vs. validation accuracy



Fig. — training vs. validation loss

Test-set performance

96.49%

TEST ACCURACY

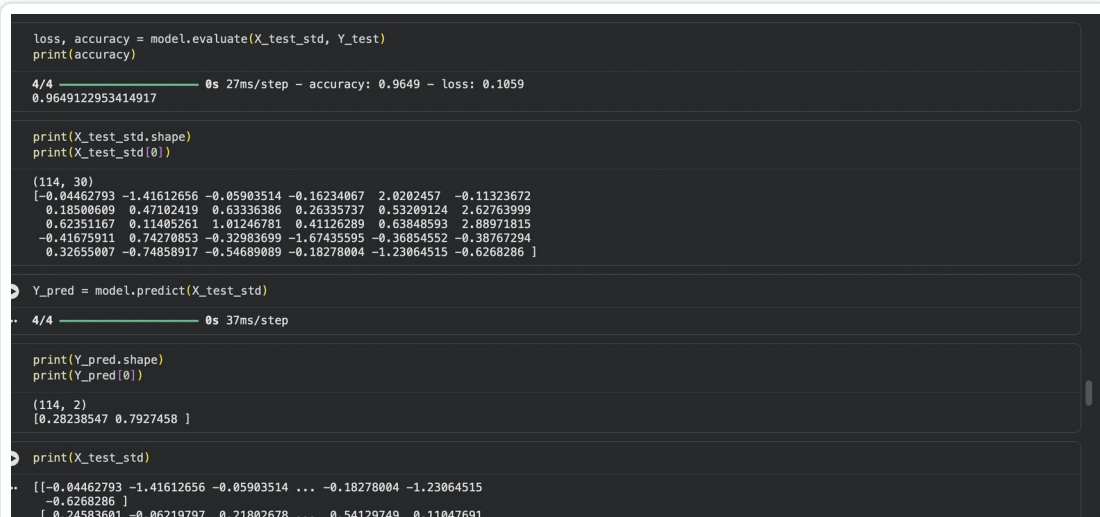
Evaluating on the 114 held-out test samples gives 96.49% accuracy with a test loss of 0.1059 — roughly 110 of 114 samples classified correctly, and a clear margin above the 90% project target. The gap between training (98.29%) and test accuracy is under two points, indicating the network learned the underlying relationship rather than memorising training records.

READ THE NUMBER CAREFULLY

- With only 114 test samples, each misclassification moves accuracy by ~0.88 points — a different random split would likely land between 94% and 98%.
- Overall accuracy is not the most important metric in diagnostics: a false negative (malignant read as benign) carries far greater consequence than a false positive.
- Sensitivity, specificity, and a confusion matrix were not yet computed — identified as necessary next steps.

Evaluation and prediction, straight from the notebook

The test-set evaluation call reporting 96.49% accuracy, and the prediction system correctly classifying a new, unseen patient measurement as benign.



```

loss, accuracy = model.evaluate(X_test_std, Y_test)
print(accuracy)

4/4 ————— 0s 27ms/step - accuracy: 0.9649 - loss: 0.1059
0.9649122953414917

print(X_test_std.shape)
print(X_test_std[0])

(114, 30)
[-0.04462793 -1.41612656 -0.05903514 -0.16234067  2.0202457 -0.11323672
 0.18500609  0.47102419  0.63336386  0.26335737  0.53209124  2.62763999
 0.62351167  0.11405261  1.01246781  0.41126289  0.63848593  2.88971815
-0.41675911  0.74270853 -0.32983699 -1.67435595 -0.36854552 -0.38767294
 0.32655007 -0.74850917 -0.54689089 -0.18278004 -1.23064515 -0.6268286 ]

Y_pred = model.predict(X_test_std)

4/4 ————— 0s 37ms/step

print(Y_pred.shape)
print(Y_pred[0])

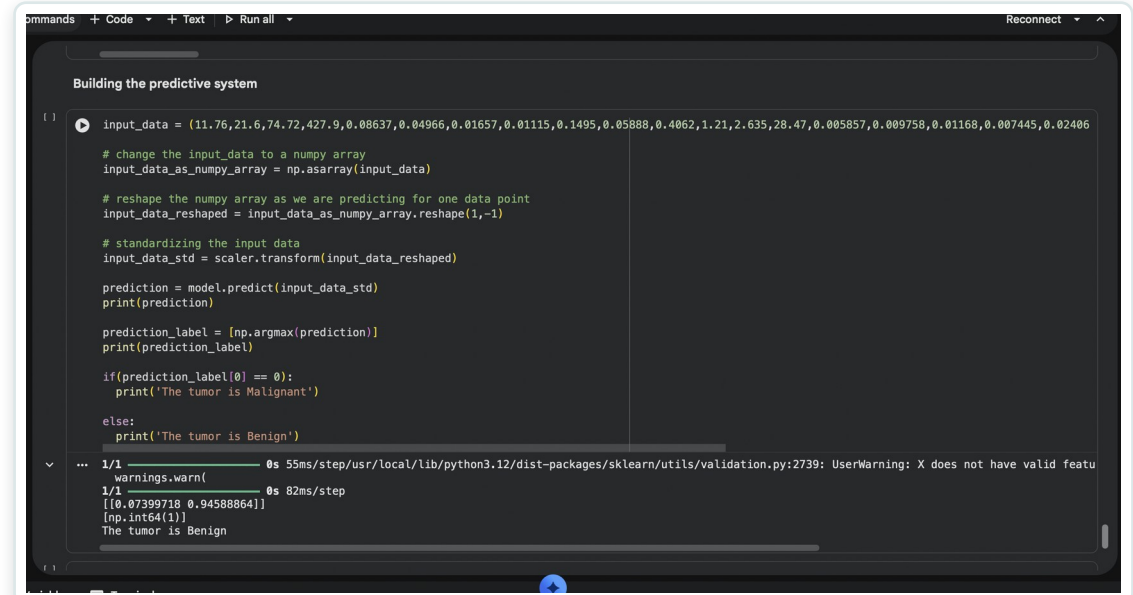
(114, 2)
[0.28238547 0.7927458 ]

print(X_test_std)

[[-0.04462793 -1.41612656 -0.05903514 ... -0.18278004 -1.23064515
 -0.6268286 ]
 [ 0.24583601 -0.06219797  0.21802678 ...  0.54129749  0.11047691
 ...]]

```

Fig. — `model.evaluate()` on the 114 test samples: 96.49% accuracy



```

Building the predictive system

input_data = (11.76,21.6,74.72,427.9,0.08637,0.04966,0.01657,0.01115,0.1495,0.05888,0.4062,1.21,2.635,28.47,0.005857,0.009758,0.01168,0.007445,0.02406)

# change the input_data to a numpy array
input_data_as_numpy_array = np.asarray(input_data)

# reshape the numpy array as we are predicting for one data point
input_data_resaped = input_data_as_numpy_array.reshape(1,-1)

# standardizing the input data
input_data_std = scaler.transform(input_data_resaped)

prediction = model.predict(input_data_std)
print(prediction)

prediction_label = [np.argmax(prediction)]
print(prediction_label)

if(prediction_label[0] == 0):
    print('The tumor is Malignant')

else:
    print('The tumor is Benign')

... 1/1 ————— 0s 55ms/step/usr/local/lib/python3.12/dist-packages/sklearn/utils/validation.py:2739: UserWarning: X does not have valid feature names: {}
warnings.warn(
1/1 ————— 0s 82ms/step
[[0.07399718 0.94588864]]
[1]
The tumor is Benign

```

Fig. — prediction system classifying a new patient as benign

What it could support — and what stands in the way

POTENTIAL APPLICATIONS

- Decision support in oncology clinics, alongside pathologist review
- Triage in diagnostic labs, flagging likely-malignant cases for priority review
- Teaching and training on quantitative diagnostic features
- Research: rapid analysis of tumour characteristics at scale

Important qualification: this is a proof of concept validated on one historical dataset. It is not a diagnostic device — real deployment would require independent validation, prospective clinical evaluation, and regulatory clearance.

LIMITATIONS

- Small dataset (569 samples, 114 test) — accuracy estimates carry real uncertainty
- Data dates from the early 1990s and one institution only
- Only accuracy and loss reported; no sensitivity, specificity, or confusion matrix
- Binary only — no subtype or stage identification
- No interpretability — the network gives no explanation for individual predictions

CONCLUSION

A sound proof of concept, honestly evaluated

The network reached 96.49% test accuracy, exceeding the 90% target, with a narrow train/test gap and steadily declining validation loss — clear signs of genuine generalisation rather than overfitting. Careful preprocessing — checking for missing values, comparing class-wise feature means, and fitting the scaler on training data only — accounted for much of the final performance, arguably more than the network architecture itself.

Within its bounds — a small, single-source, historical dataset evaluated on accuracy alone — the project supports a clear conclusion: neural networks are well suited to structured medical classification problems, and with careful preprocessing and honest evaluation they can achieve strong, reproducible results.

NEXT STEPS

- Report confusion matrix, sensitivity & specificity
- Apply k-fold cross-validation for a reliable accuracy estimate
- Validate on independent, contemporary, multi-institution data
- Apply SHAP for prediction-level interpretability
- Explore ensembles with tree-based classifiers
- Extend to multi-class subtyping and multi-modal (imaging) input



Thank You

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